

Inferential Statistics: Knowledge Score vs. Actual Practices

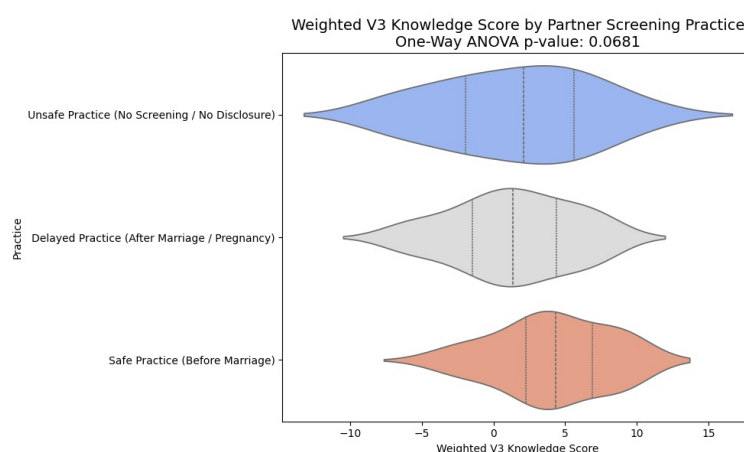
This report examines whether clinical knowledge dictates the actual actions a carrier takes regarding family planning (Partner Screening) and cascade screening (Family Disclosure).

1. Partner Screening Practice (One-Way ANOVA)

We grouped the participants into three behavioral tiers: *Safe* (Screened before marriage), *Delayed* (Screened after marriage/pregnancy), and *Unsafe* (Did not screen/disclose). Note: Participants marked 'Other' (usually single/unmarried) were excluded.

Behavioral Tier	N	Mean (Weighted V3)	Std Dev (Weighted)
Unsafe Practice (No Screening / No Disclosure)	28	1.583	5.299
Delayed Practice (After Marriage / Pregnancy)	34	1.342	4.097
Safe Practice (Before Marriage)	21	4.105	3.822

- **F-Statistic:** 2.779
- **P-Value:** 0.0681
- **Conclusion:** Not Statistically Significant ($p > 0.05$). Clinical knowledge does not significantly alter actual partner screening practices.



2. Family Disclosure Practice (Welch's T-Test)

Comparing carriers who disclosed their status to their family versus those who kept it a secret.

Disclosed to Family?	N	Mean (Weighted V3)	Std Dev (Weighted)
No	15	0.375	6.606
Yes	182	2.139	4.378

- **T-Statistic:** 1.016
- **P-Value:** 0.3258
- **Conclusion:** Not Statistically Significant ($p > 0.05$). Clinical knowledge does not significantly influence whether a carrier tells their family.

